SAINT MICHAEL COLLEGE OF CARAGA

THE PERCEIVED EFFECTIVENESS OF DRUG REHABILITATION AND AFTERCARE PROGRAMS

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TABLE OF CONTENTS

		Pages
TITLE PAGE		i
CHAPTER		
1	**THE PROBLEM AND ITS SCOPE**	2
	1.1 Background of the study	2
	1.2 Review of Related Literature	4
	1.3 Theoretical Framework	18
	1.4 Schematic Diagram	21
	1.5 Statement of the Problem/ Objectives of the Study	22
	1.6 Hypothesis	22
	1.7 Definition of Terms	23
2	**RESEARCH METHODOLOGY**	25
	2.1 Research Design	25
	2.2 Research Materials and Methods	25
	2.3 Research Environment, Population, and Sample	26
	2.4 Research Data and Analysis Plan	27
	2.5 Research Ethical Standards	28
REFERENCES		33

CHAPTER 1

THE PROBLEM AND ITS SCOPE

1.1 Background of the study

Drug use is one of the big problems facing both the Philippines and other countries around the world. People use drugs for many reasons: they want to feel good and relax, stop feeling bad, perform better in school or at work, or are curious because others are doing it, and they want to fit in. Most drugs are known to bring happy feelings and often lead to harmful changes in behavior, especially at parties and when people are under stress. Moreover, drug use can result in countless crimes, financial problems, and broken families.

According to the United Nations Office on Drugs and Crime (2024), the number of people who use drugs has risen to 292 million in the past year, a 20 percent increase over 10 years. However, in the Philippine context, according to the Dangerous Drug Board on the Barangay Drug Clearing Program Report (2024), nationwide data identified a total of 450,253 persons who use drugs (PWUDS), which affects not only the person who uses drugs but also their families and the community. In response to the growing number of (PWUDS), the government and other non-government agencies established drug rehabilitation and aftercare services that provide treatment and strategies to prevent relapse of individuals recovering from substance use disorders, and reintegration support (Dangerous Drug Board, Regulation No. 1 Series of 2006).

Drug rehabilitation and aftercare programs are very different from one country to another, as they match the diverse cultural, social, and health care settings. Many countries use special drug rehab programs made for their own artistic and social ways. Costa Rica offers affordable rehab options, often situated in natural settings, and provides a comprehensive

approach to addressing physical, mental, and spiritual harm. It relies heavily on community support for recovery programs. In the same way, Australia has focused more on a public health approach to handle drug addiction, with many services like outpatient and living rehab programs, and aftercare services, through which they also focus on and support research with proven drug addiction treatments (Desert Hope Treatment Center, 2025). In the Philippines, community-based and therapeutic community (TC) have replaced punitive war on drugs strategies. The Department of Health initiated the INTREPRET program, which combines cognitive-behavioral treatment and family learning sessions, to reduce the risk of relapse (Kanamori et al., 2024). The Dangerous Drug Board also brought in an aftercare program to facilitate clients' reintegration into the community and to prevent relapse to drug use. This shows that drug addiction programs vary across countries, with each emphasizing culture and community participation in rehabilitation.

However, many countries have a successful rehabilitation, but they have also faced challenges. In Malaysia, rehabilitation programs have tried to fight drug addiction and boost self-growth, but they are criticized for not working well in stopping relapse. Several factors hinder the program's success, including the quality of the rehabilitation lessons, the number of inmates participating, and the availability of after-release services (Zakaria et al., 2025). In Vietnam, however, there are chances and problems within the drug rehab system in terms of leadership and rule-making, money, workers, info systems, and service giving for people with SUD. They found big blocks caused by the lack of clear directions for helpful rules, not enough money for community-based services, ongoing bad views from helpers toward people with SUD, and the lack of community-based drug treatment other than methadone (Giang et al., 2024).

Also, in South Africa, they encountered challenges in providing aftercare services. Service helpers face numerous system problems, and rules that require aftercare services will help strengthen the aftercare system. The aftercare system and service delivery may be strengthened and sustained through an aftercare model that is family-focused, culturally aware, integrated into current services, and encourages helper engagement (Mpanza et al., 2022). The USAID Renew Health project in the Philippines demonstrates that this approach is cost-effective and less expensive than inpatient treatment. In a multi-case study with five LGUs, where CBRP was highly developed and very good, especially in the health and mental health parts, group-agency teamwork was also evident (Hechanova et al., 2022).

However, most research has focused on the facilities' challenges and successes, while few studies have explored how facilitators/implementers and clients view the effectiveness of these programs. Understanding their perspectives is important because they directly manage, deliver, and benefit the rehabilitation and aftercare services. Therefore, this study aims to determine facilitators' and clients' perceptions about the effectiveness of drug rehabilitation and aftercare programs, particularly in terms of program implementation, social reintegration, and relapse prevention.

The study may benefit the government, particularly the Department of Health (DOH), the Dangerous Drugs Board (DDB), and local government units, by identifying operational gaps and providing insights to enhance program standards, policies, and resource allocation, and improving rehabilitation services. The rehabilitation center may also benefit from this study to improve its program design and implementation, social reintegration support, and relapse prevention strategies. The findings may also benefit communities and clients by demonstrating the effectiveness of rehabilitation in promoting long-term recovery, reducing

relapse rates, supporting successful reintegration, and thereby fostering safer, healthier communities.

1.2 Review of Related Literature

This section provides an overview of studies and writings related to our research. The researchers utilized multiple sources to inform this study.

Drug recovery is a way to get better from drug addiction. It includes medical help and talks for those who find it hard to stop using drugs. The main objective is not just to eliminate drug use but also to help with the emotional, social, and psychological aspects of addiction. Treatment can include talking to someone, taking medication for problems like depression, and helping with an individual's overall well-being. Rehab centers offer a range of services, including in-house care, outpatient treatment, living in a recovery house, long-term support, and participation in self-help groups. It is important to note that there is no one treatment for all. Everyone needs a plan that fits their own needs.

Aftercare is the support provided immediately after someone leaves a rehab center (Sereta, 2016). It helps people use the skills they learned in rehab in their daily life with support from the center. Aftercare is key for anyone getting better from drug addiction because it boosts their chances of staying clean. Aftercare helps individuals adjust to everyday life and reduces the likelihood of requiring further treatment. It also helps address problems that arise while staying clean and working on improvement (UNODC, 2025).

Also, aftercare is a kind of social help program that aims to support specific groups in need, like those who were in jail, drug users, troubled kids, and people with disabilities,

through advice, talks, things they need, and/or moral support. According to Anser et al. (2014), aftercare programs are designed to reestablish family ties, reintegrate individuals into society, and facilitate a smoother transition to everyday life after a prolonged absence.

Additionally, aftercare strategies, including relapse prevention, coping skills training, and participation in self-help groups, have been demonstrated to help individuals stay in treatment longer. These strategies also reduce relapse and criminal behavior while increasing employment (Daigre et al., 2021). According to Malinao (2024), aftercare programs are essential for people who are trying to stop using drugs. These programs help people as they move from special treatment back to their everyday lives, so they can stay healthy and avoid using drugs again. Aftercare programs also teach people how to say no to drugs, even in challenging situations.

Nevertheless, good aftercare programs can offer many benefits that significantly support a person's recovery, such as ongoing support and regular follow-ups to help them face challenges after treatment. Ways to stop relapse and handle tough times, people will learn good ways and plans to avoid relapse, keeping them on the path to recovery and also fitting into daily life and the community, which will help the person join society and everyday routines again and also build strength and independence and lastly to offer long-term recovery success, it dramatically boosts the chances of staying clean for a long time by lowering relapsing chances, key to keeping sober and winning in the long run (Pathways, 2024).

The Dangerous Drug Board stated that aftercare and follow-up are an integral component of the treatment and rehabilitation process. It is a continuation of the rehabilitation process within the community after discharge from a treatment facility,

imparting new skills for sustaining recovery, including handling everyday responsibilities and managing family, employment, and livelihood programs. The aftercare and follow-up program facilitates the client's reintegration into the community and prevents relapse into drug dependency.

LENGTH OF SERVICE

According to Madeleine et al. (2019), long-term work enables the development of a deeper understanding of the group's needs, allowing them to refine their methods and achieve better teamwork over time. Staff with more experience are more likely to utilize a broader range of proven approaches; their experience also enhances the perceived meaningfulness of the treatment and makes it easier to implement (Kraan et al., 2018). Similarly, peer recovery helpers with more work time demonstrate significantly better skills in engaging people after training (Anvari et al., 2022). Manuel et al. (2016) state that steady help during aftercare was key to success in joining back into life after living treatment, since support breaks often led to relapse and worse results. Additionally, Hechanova et al. (2022) observed that steady, long-term helpers in community-based rehabilitation programs in the Philippines experienced better health, social, and behavioral outcomes than those with staff turnover. Soriano et al. (2024) found that staff in healing community rehabilitation centers developed empathy, respect, and strong connections over time, demonstrating the importance of experience and consistent support in sustaining recovery.

A study by García-Huidobro et al. (2021) found that programs with longer staff involvement (12 months or more) resulted in greater client treatment success than those with shorter involvement periods. Also, Polcin et al. (2024) found that individuals in sober living

homes performed best when guided by staff with more years of experience, which helped them maintain sobriety and achieve their recovery goals.

However, according to D'Ippolito et al. (2015), one of the workforce challenges in addiction services is maintaining trained facilitators over time, with staff who have been in the role for more extended periods being more capable of implementing evidence-based practices effectively and building trust with clients.

In contrast, studies show that new counselors just starting their careers can be just as hands-on, or even more effective, than veteran counselors with decades of experience. A large-scale study of 170 therapists and over 6,600 clients found that therapists do not automatically improve with more experience (Goldberg et al., 2016).

TRAINING BACKGROUND

Rehabilitation workers in drug rehab centers play a significant role in improving patient results and care quality. Additionally, training is key to the success and effectiveness of drug rehab and aftercare programs. However, a full review by Madson et al. (2018) revealed that set training enhanced staff skills and treatment loyalty, although differences in training time were observed, and both problems and follow-ups impacted long-term results.

Ongoing training ensures that staff possess the necessary knowledge and skills to deliver high-quality care to individuals undergoing addiction treatment. Additionally, it helps individuals follow rules and laws while enhancing their job skills and expertise. Training also helps introduce new approaches and ideas for treatment, enabling centers to stay current with the latest developments in addiction care. Drug rehabilitation facilities can reap numerous benefits by investing in ongoing training, including improved patient outcomes,

enhanced staff confidence and job satisfaction, a stronger facility reputation and trust, and the ability to stay current with field changes, including advances and best practices (Faraz, 2023).

Similarly, staff members who are sufficiently trained, hold multiple degrees, diplomas, and certificates from reputable institutions, and demonstrate kindness, patience, friendliness, respect, and warmth inspire trust in participants throughout treatment. It significantly impacts the effectiveness of rehabilitation (Mustapha et al., 2023; Sereta et al., 2016).

A Systematic Review and Meta-Analysis by Zhao et al. (2024) found that training has a positive impact on enhancing patients' self-care abilities, treatment outcomes, and satisfaction. Also, training combines educational methods and management support to overcome implementation barriers and optimize the benefits of clinical practice. Moreover, staff who attend multiple training and seminars gain more knowledge about substance use-related issues, reduce stigma, change their attitudes, and improve working skills among the different services providing care to users (Domingues et al., 2023).

Staff who join training workshops build their drive to help drug users get better through training. They learned how to help recovering users better, and they knew how to use some tricks, and also changed their views toward drug users, which led to significant improvements in drug recovery for the users (Hechanova et al., 2019). The Department of Health's Community-Based Rehabilitation Program (CBRP) and the DSWD Yakap Bayan Program encompass a range of building-up efforts, including training staff to help run and provide aftercare services. Staff with a more extensive training and education background tend to view rehab and aftercare programs in a positive light, especially regarding how they work and handle relapse.

PROGRAM IMPLEMENTATION

Drug addiction is a global public health problem affecting individuals, families, and communities. Rehabilitation programs play a key role in addressing this crisis. They provide structured environments that are focused on recovery, enabling individuals to rebuild their lives. The success of such programs relies heavily on facilitators, professionals, and trained personnel who guide, support, and monitor participants during recovery (Hechanova et al., 2019).

Reviewed studies by Louie et al. (2021) on addiction treatment. They found that while many programs claimed to use evidence-based methods, only a few actually measured whether proper implementation led to better client outcomes.

Drug rehabilitation implementation has indeed shown that the involved factors, including intrinsic motivation, social support, and fulfilment of psychological needs during the rehabilitation process, have influenced the successful implementation of drug rehabilitation programs (Erliyani et al., 2024). Additionally, Narkeyah et al. (2025) also identified multiple factors that influence the effectiveness of the program, including the quality of the rehabilitation modules, the level of patients' engagement, and the availability of post-release support services, which can increase the effectiveness of the programs that enhance the prevention of a person from returning to drug use.

Cerna et al. (2024) present a study about the challenges in rehabilitation faced by workers in the aftercare process of substance abuse. The study revealed various challenges faced by workers during implementation, including workforce shortages, clients' commitment

to aftercare, handling deception, unforeseen circumstances, maintaining professional roles, and limitations among existing workers.

In Dagupan, a drug treatment and rehabilitation center, researchers identified two factors that affect the aftercare of drug dependents. The internal factors, which are emotional and socio-economic factors, include experiencing a persistent feeling of deep sadness, having negative thoughts, and experiencing physical abuse that may affect the recovery, which may lead to relapse. While the external factors that affect the aftercare of drug dependents include unemployment and poverty, which lead to relapse, and drug dependents have experienced a loss of interest in everyday activities, persistent feelings of sadness, and have experienced physical or emotional abuse that affects their recovery (Samson, 2021).

However, the study of Bondad (2020) found that the aftercare intervention helped maintain abstinence, increase refusal, enhance psychological flexibility, and improve emotional and psychological well-being, suggesting its potential effectiveness in preventing substance use relapse. Furthermore, it provides a change in path for individuals, particularly in their personal lives, as they can change their wrong behaviors and practice abstinence and avoid the influence of drug use. However, there are challenges in the aftercare of reintegrated individuals, such as unemployment and societal stigma. This emphasizes the need for continuous implementation of aftercare programs to guide and support them in reentry (Valencia, 2023).

The study by Acuna et al. (2024) on the effectiveness of the community-based rehabilitation program in Capiz found that the program's implementation was very effective, particularly in livelihood training, counseling, health and wellness activities, and lastly, educational and awareness Programs. Moreover, the program reveals challenges despite its

effectiveness, including limited resources and rehabilitation centers, insufficient support from the local government, and a shortage of staff for counseling and conducting seminars.

Also, according to De Costa (2019), drug treatment cannot be effective if it treats only one aspect of an individual; it involves and affects the whole person, and a single person comprises biological, psychological, social, and spiritual aspects, and each person is made up of the same elements. However, their individual needs may vary, and it is ineffective when drug-related deaths and relapse rates increase. On the other hand, when treatment is available, it attends to a person's needs and provides individualized care tailored to each individual's specific requirements.

In the study by Damayon et al. (2023), the Community-Based Drug program of the Philippine National Police (PNP) in Batangas had clear plans and was well-known to participants. However, the program still needs to improve its activities and the way the community gets involved. Problems such as limited resources and gaps in program management make it harder to achieve good results.

Trinidad et al. (2023) studied the effectiveness of a rehabilitation program for people in the community who are drug-dependent. They found that the program works well and that many participants are satisfied with it. However, the study also revealed some problems faced by those in rehab. These include loss of income during treatment, not enough food, difficulty understanding some seminar topics, feeling bored and losing focus during lectures, varying levels of family support, and feeling unwanted or rejected by friends and the community because of their drug-dependent status.

The study by Almadrones and Babala (2024) found that the Drug Rehabilitation Program in Basud, Camarines Norte, is a well-run rehabilitation center that excels in advocacy,

screening, and treatment. However, follow-up care and inpatient treatment are less common and have some gaps. The research emphasizes the need for ongoing community training and education to improve support and help people recover from drug use for the long term.

RELAPSE PREVENTION

Most drug treatment and rehabilitation centers teach relapse prevention skills that help clients live a happy life after recovery. These skills support maintaining recovery, reaching long-term goals, and coping with challenges to prevent relapse (Parisi, 2025).

Relapse prevention is a cognitive-behavioral method that helps people recognize and deal with situations that might lead to relapse. It supports individuals in recovery by assisting them in noticing early warning signs and learn coping skills to avoid relapse, especially during the early stages when the chance of staying sober is highest (Malinao, 2024).

According to NIDA (2020), boredom and stress are two common reasons that people relapse into drug use. Kabisa et al. (2021) found that relapse after detox and rehabilitation is a global public health problem. About 40 to 60 percent of people relapse after treatment for substance use disorder. Risk factors include family conflicts, psychological stress, pressure from friends, and social and economic issues like easy access to drugs, influence from peer groups, and lack of confidence to say no.

A systematic review by Erliyani et al. (2024) found that relapse prevention programs have a significant role in enhancing the effectiveness of rehabilitation efforts. How well a center prevents relapse shows how effective it is at helping people avoid returning to drug use. Kandasamy and Menon (2018) explained that relapse prevention has two main goals: stopping the first slip-up from happening and helping people stay drug-free or reduce harm.

If a slip-up does happen, relapse prevention also helps manage it to stop more relapses from following.

There are four main types of relapse prevention. First, relapse is a slow process that happens in stages, starting with minor warning signs before drug use begins again. Second, recovery involves personal growth and reaching important milestones. Third, the primary tools for preventing relapse are cognitive therapy and mind-body relaxation, which help change negative thoughts and develop healthy coping skills. Fourth, most relapses follow a few basic patterns divided into three types: physical, mental, and emotional (Melemis, 2015). Understanding these types helps both patients and doctors see how social support can play an important role in avoiding drug use again.

Harada et al. (2023) found that factors like low education attainment, unemployment, having alcohol abuse problems, and other health issues increase the risk of relapse. Kabisa et al. (2021) also found that 59.9% of people relapsed after treatment. Some common risk factors for relapse include family conflicts, pressure from friends, easy access to drugs, lack of stability, being a parent, how long someone stayed in the hospital, how much substance they used, family stress, and living conditions.

Rehabilitation is essential in helping people avoid relapse by changing their behavior and providing ongoing support (Erliyani et al., 2024). Chan et al. (2019) found that personal qualities like self-growth, caring for others, spirituality, and religious beliefs also help people stay sober and prevent relapse after rehab. Together, these studies demonstrate that both a well-organized rehabilitation program and an individual's inner strengths are crucial for maintaining recovery and preventing relapses.

SOCIAL REINTEGRATION

Reintegration is the process of returning to society after finishing a prison sentence or a drug treatment program. It means settling back into everyday life in a way that reduces isolation, stigma, and financial hardship, helping people rejoin the community (Beaudry et al., 2021).

Many people see drug addiction as a moral failure instead of a treatable health condition. This view significantly affects how well former drug users can reintegrate into society. A former drug user often faces two types of stigma: positive and negative. Some people have started to notice and appreciate the positive changes in former addicts after rehabilitation, such as becoming more polite, being more independent, and avoiding bad behavior. However, negative stigma remains strong, especially when problems occur in their community (Saputra et al., 2025).

Widoyoko et al. (2024) also studied the legal measures that support the social reintegration of drug users, focusing on rehabilitation as a way to help people change. In-prison programs help prepare individuals to return to society as productive members. Despite the existence of laws, policies, and numerous support programs, the effects of social stigma, inadequate enforcement, and a lack of resources have limited their impact. Policies should adhere to international standards, particularly as community programs expand, public awareness increases, and more effective monitoring systems are implemented. These steps are needed to create a supportive legal and social environment that helps drug users successfully reintegrate into society.

According to Widoyoko et al. (2024), successful drug rehabilitation and reintegration require laws grounded in public health, inclusiveness, and social justice. The social

reintegration of people who have finished drug treatment is strongly affected by family, social structure, job opportunities, marital status, and stigma. The study also found that being married can help increase the chances of successful reintegration and recovery, as shared by people in rehab. Additionally, stigma is experienced not only in the community but also at work and even among friends and peers.

Mayock and Butler (2021) studied the experiences of long-term methadone clients in Dublin, Ireland, to understand the challenges they face when trying to reintegrate into society, even though there are policies to support them. Similarly, Gideon (2017) examined the experiences of former inmates in Israel who underwent rehabilitation. He found that while families can support reintegration, unresolved conflicts and unmet expectations at home can sometimes affect the recovery process.

De Siqueira et al. (2015) did a study about the family of a person addicted to crack who was in detox treatment. They found that the family plays a vital role in helping their relative reintegrate into society. Families share responsibility for the treatment and help guide their loved one toward a new path. They know they need to be present in the life of the person struggling with addiction, always looking for ways to support and help them deal with their pain and recovery.

Ghuloom (2021) studied the social challenges that people who have completed drug rehabilitation face in society. The research revealed that numerous barriers hinder their ability to reintegrate. These include negative social stigma, discrimination, and a lack of job opportunities or problems with access to rehabilitation services. By highlighting these social barriers, the study helps people understand these issues better and encourages them to take responsibility and act to support those in recovery.

According to Ahmet Kılıç and Tuysuz (2024), having a job is very important for people who have finished a drug rehabilitation program to recover fully and reintegrate into society. Many people face significant challenges, like employers mistreating them because of their past drug use, lacking job skills, having a poor work history, and dealing with social stigma. Psychological issues, legal restrictions, and losing access to support can also make it hard for them to return successfully to society. Despite these difficulties, having a good job provides stability, a sense of purpose, and social connections. These things help strengthen recovery, boost self-worth, and increase happiness (Rumrill & Bishop, 2023; Stallvik et al., 2019; Johannessen et al., 2019; Kewley & Hout, 2020; Sahker et al., 2019).

According to Morrison et al (2024), Many people who complete drug rehabilitation and aftercare still struggle to secure stable employment, a challenge that undermines the perceived effectiveness of rehabilitation and aftercare, because long-term recovery depends not only on abstinence but also on successful social reintegration. For example, a review by the European Monitoring Centre for Drugs and Drug Addiction (EMCDDA) noted that drug-treatment alone, without added support for education, vocational training, or employment services, generally has “only limited and inconsistent effects on employment outcomes.”

Moreover, a study of unemployed drug users found that many face significant barriers, such as low interpersonal or technical (e.g., computer) skills, which reduce their employability compared with general job seekers. Because of these structural and social barriers, recovering individuals may remain unemployed or under-employed despite completing rehabilitation, which in turn casts doubt on the overall success and societal benefit of drug rehabilitation and aftercare programs if they fail to enable meaningful reintegration into work and stable income.

The dramatic growth in the number of adults under criminal justice system control in the US has been found to have a wide range of negative public health consequences. Researchers have found that communities with high rates of incarceration have poorer health, less access to education and employment, and shorter life expectancies. Recent research has begun to explore whether these incarceration effects are also seen with community-supervised populations. The findings indicate that those on probation carry a high health burden, that they experience distinct disadvantages in the labor market associated with this health burden, and that protective factors such as education and income offer less protection. We believe that these points to the importance of holistic approaches to services for this population. For example, mental and behavioral health assessment and support services may need to be integrated into employment-specific services for employment services to be effective, and employment training and support services may need to be integrated into mental and behavioral health services. Given the complex health needs of this population, it will be important for policymakers, researchers, and practitioners to look outside the criminal justice field for guidance on responding to the service needs of this population (e.g., nursing, psychiatry, addiction) and to avoid repeating the harms of mass incarceration.

In addition, Lilja, M. (2019) Many participants in drug rehabilitation, especially those with histories of substance use and incarceration, report that securing employment after release is extremely difficult, a barrier that deeply colors their view of whether the rehabilitation and aftercare program truly helped them. For instance, in the 2019 study "Formerly imprisoned polydrug users' narratives about unemployment," many ex-users described persistent rejection from employers and chronic unemployment despite their efforts to reintegrate. Because having a stable job is often seen as a key marker of "getting

back to normal," these employment struggles may lead them and others around them to judge the rehabilitation less positively, even if they have maintained abstinence or improved personally. In other words, when aftercare does not translate into socially and economically meaningful outcomes (like stable employment), people may feel the program was only partly effective, undermining confidence in long-term recovery support.

Employment is a crucial component of recovery capital. Recovery capital refers to the resources and strengths individuals use to support their recovery and maintain their health. Having a job not only gives financial stability but also helps people build a new identity and connect with others (Sahker et al., 2019). Activities such as earning money independently and maintaining a daily work routine help people stay on track with their recovery (Frone et al., 2022). However, past research indicates a lack of information on how people in recovery find employment, and further studies are needed to understand this process better (Robertson et al., 2021).

1.3 Theoretical Framework

The study is based on the Recovery Capital Theory by Granfield & Cloud (1999) and the Transtheoretical Theory of Behavior Change by Prochaska & DiClemente (1983).

Transtheoretical Model

The Transtheoretical Model (TTM), developed by Prochaska and DiClemente (1983), posits that behavioral change occurs through a series of progressive stages: precontemplation, contemplation, preparation, action, and maintenance. This theory applies

to the study, as it explains how clients' readiness to change affects their views of program implementation and efficacy. Facilitators who understand the client's stage of development are better able to implement stage-matched interventions, which may lead to greater perceived program efficacy. Factors such as length of service and training are said to influence their evaluations of client improvement and program efficacy (DiClemente & Crisafulli, 2022).

According to Raihan (2023), developing a reliable framework for self-change, or professionally assisted change, is beneficial for both individuals seeking change and those in the healthcare system who help with it. When using the model of change, "changers" are not coerced but instead supported and accepted at the stage they are at present. Treatment is tailored to each individual's specific stage of disease. With this method, "change behavior" is viewed as a fluid, dynamic process with the potential for recycling stages, rather than focusing on the end goal of change. The importance of discussing the common and potential risks of relapse allows the focus to turn from a failure mindset to the success achieved up to the point of relapse. Allowing the discussion of relapse in a nonpunitive manner leads to an honest examination of lessons learned regarding the individual, their triggers, and better ways to address those triggers. Overall, the Transtheoretical Model helps improve compliance and motivates people toward the prospect of change.

Recovery Capital Theory

The notion of Recovery capital has been developed by authors such as Robert Granfield and William Cloud, who have applied the concept of capital to recovery-oriented scholarships. They devised the term recovery capital to describe the breadth and depth of internal and external resources that an individual can draw upon to initiate and sustain

recovery from substance use (Granfield and Cloud, 1999). The term, first applied in the book Coming Clean: Overcoming Addiction Without Treatment, is based upon 46 semi-structured interviews with individuals who recovered from substance use without any professional help and/or support. A process known as natural recovery. The authors explored the social context of these individuals' natural recovery and the circumstances that facilitated their journey from substance use.

Granfield and Cloud (1999) conceptualize recovery capital as a set of four interrelated but distinct forms of resources that influence an individual's ability to recover from substance use: social, physical, human, and cultural capital. Social capital arises from a person's relationships and social environment, providing access to support, shared resources, and mutual obligations that can assist in life improvement and recovery. Belonging to supportive social networks expands an individual's choices and strengthens their capacity to sustain recovery.

Physical capital refers to material and financial resources such as income, savings, property, or other assets. Financial stability increases recovery opportunities by allowing individuals greater flexibility, including time away from work, access to treatment, or relocation when distancing from substance-using environments is necessary (Granfield and Cloud 1999).

Human capital encompasses personal attributes and abilities that enable individuals to function effectively in society. This includes education, health, emotional wellbeing, coping skills, self-confidence, interpersonal abilities, and a sense of responsibility. These qualities

help individuals manage daily challenges and adapt to new, healthier patterns of behavior (Granfield and Cloud 1999).

Cultural capital relates to the values, beliefs, attitudes, and norms individuals acquire through their social and cultural affiliations. Those who remain connected to conventional social expectations and norms tend to have advantages in recovery compared to individuals who have been socialized to reject mainstream values (Granfield and Cloud 1999).

Recovery capital both shapes and is shaped by the recovery process. The amount and type of capital an individual possesses directly affect the strategies available to them and the likelihood of successful recovery from substance use.

Moreover, Best and Laudet (2010) have identified and developed three clusters of recovery capital – personal, social, and community – as dynamic influences in the growth of well-being and recovery. Since then, academics such as White (2008) have developed the concept to describe observed changes in substance users' resilience and robustness of social and emotional circumstances in long-term recovery. Social capital is seen as a critical determinant of building personal strengths and resources, and for tapping into community supports and resources. Additionally, Yates (2015) has illustrated how recovery capital can be used as a means of avoiding relapse, sustaining recovery, and encouraging change.

1.4 **Schematic Diagram**

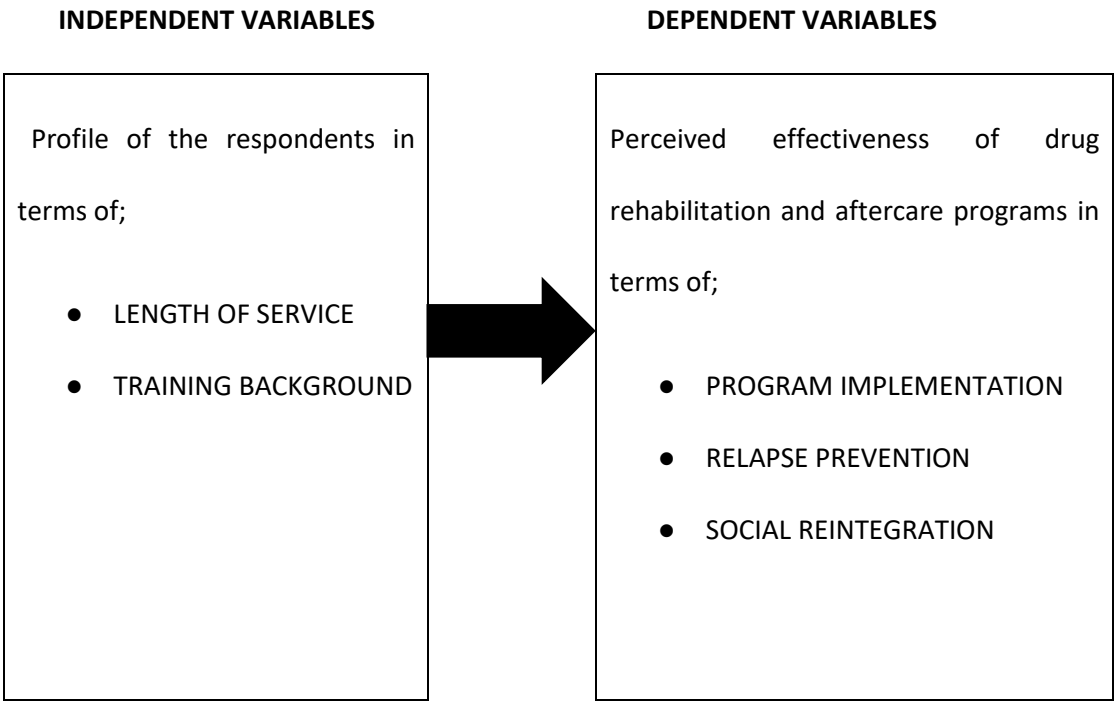


Figure 1. The schematic diagram shows the relationship between the independent and dependent variables.



1.5 Statement of the Problem

This study aims to determine the effectiveness of drug rehabilitation and aftercare programs as perceived by facilitators. Specifically, the study seeks to answer the following questions:

1. What is the profile of the respondents in terms of;

   1.1 Age;

   1.2 Length of Service; and

   1.3 Training Background?

2. What is the level of perceived effectiveness of drug rehabilitation and aftercare programs in terms of;

   2.1 Program Implementation;

   2.2 Social Reintegration; and

   2.3 Relapse Prevention?

3. Is there a significant relationship between the respondents' profiles and their perceived effectiveness on drug rehabilitation and aftercare programs?

1.6 Hypothesis

*H*o: There is no significant relationship between the respondents' profile and their perceived effectiveness of the drug rehabilitation and aftercare program.

1.7 Scope and Limitations

This study focused on the effectiveness of drug rehabilitation and aftercare programs from the perspective of the clients and facilitators or persons directly involved in implementing the programs at the Drug Treatment and Rehabilitation Center in Surigao del Norte during the academic year 2025-2026. using quantitative surveys to collect data, considering variables such as Program implementation, relapse prevention, and social reintegration. The researcher will focus solely on facilitators and clients; it will not cover private facilitators, policymakers, or other stakeholders. Furthermore, the study's findings are not fully generalizable to all drug rehabilitation and aftercare programs due to the limited sample size.

1.8 Definition of Terms

The following terms used in this research are operationally and conceptually defined.

Aftercare program. Refers to ongoing support provided after completing the earlier phases of treatment and rehabilitation.

Drug Rehabilitation. It is a program designed to help individuals with substance use disorders stop using drugs, manage withdrawal symptoms, develop coping skills, and restore healthy functioning.

Effectiveness. It measures how well drug rehabilitation and aftercare programs meet their objectives in helping clients recover, reintegrate, and sustain sobriety.

Facilitator. Refers to someone who guides, supervises, and delivers programs and interventions, helping participants achieve the program's objectives

Length of service. Refers to the time an individual has been employed or has served in a specific role within an organization

Perceived. Refers to the respondent's personal judgment on the effectiveness of drug rehabilitation and aftercare programs based on their own experiences and observations.

Relapse Prevention. Refers to a treatment strategy that helps clients avoid returning to substance use.

Social reintegration. It is a process through which a person who has been in rehabilitation gains freedom and returns to live among other members of society.

Substance abuse. Refers to the excessive use of alcohol and illegal drugs.

Training Background. Refers to the workshops, certifications, or specialized courses a facilitator has completed to equip them for delivering rehabilitation and aftercare programs.

CHAPTER 2

RESEARCH METHODOLOGY

This Chapter discussed the research method, research design, research environment, research materials, the population of respondents, ethical standards, and the statistical tools used to analyze and interpret the data.

2.1 Research Design

The researcher will use a descriptive-correlational quantitative research design. The descriptive aims to describe the respondent's profile and the perceived effectiveness of drug rehabilitation and aftercare programs, focusing on program implementation, relapse prevention, and social reintegration. The correlational part aims to determine the significant relationship between the facilitators' profile and their perceived effectiveness in drug rehabilitation and aftercare programs. A quantitative research method was employed to collect and analyze numerical data (Bhandari, 2020).

This research design aims to describe the relationship between variables without implying a cause-and-effect relationship. This research design enables the measurement of the degree of association between two variables and the formulation of informed predictions based on the identified relationships (Bhandari, 2021). Applying this method could help gain an accurate, deep understanding to conclude the study.

2.2 Research Materials and Methods

A quantitative research method was employed to collect and analyze numerical data, identify patterns, test relationships between variables, and make predictions

(Bhandari, 2020). The researcher utilized a self-developed survey questionnaire as the primary research instrument. Since the questionnaire is researcher-made, it will undergo reliability and validity testing to ensure the accuracy and relevance of the study results. The questionnaire consists of two parts: the first part is the respondent's profile (length of service and training background). The second part examines the perceived effectiveness of drug rehabilitation and aftercare programs in terms of program implementation, Social reintegration, and Relapse prevention. Participants will respond using a Likert scale from 4 to 1, featuring closed-ended questions from "Strongly Agree" (SA) to "Strongly Disagree" (SD).

2.3 Research Environment, Population, and Sample

The research study will be conducted in the DOH-TRC Caraga-Drug Treatment and Rehabilitation Center, located in Barangay Anomar, Surigao del Norte. The rehabilitation center was created under the Republic Act No. 9165, also known as the "Comprehensive Dangerous Drugs Act of 2002,". DOH-TRC centers are government-run facilities supervised by the Department of Health (DOH) that provide not only residential treatment but also outpatient clinics and aftercare services for persons who use drugs (PWUD) and other substance abusers. It is a well-known rehabilitation center in the Caraga region, established in 2010.

The researcher employs a purposive sampling technique to select participants who are more likely to provide relevant and helpful information. In purposive sampling, also known as judgmental or expert sampling, participants are chosen based on the researcher's judgment and those most likely to provide relevant and helpful information (Ahmed, 2024).

Respondents	Population	
	F	%
Facilitators	45	45.0
Clients	60	60.0
Total	100	100.0

Table 1. Respondents of the Study

The table presents the number of respondents from the drug treatment and rehabilitation center in Caraga. A total of 100 participants will be involved in the study, including facilitators and clients from the drug treatment and rehabilitation center in Caraga.

2.4 Research Data and Analysis Plan

The researchers will first seek permission from the DOH-TRC treatment and rehabilitation center. After obtaining the list of respondents, the researchers will carefully plan the data-gathering process, ensuring that participants' rights, safety, and the benefits of the study are clearly stated.

Then, the researchers will provide respondents with an informed consent form that explains the study's purpose, procedures, and confidentiality measures. The participants will be allowed to review and sign the consent form, indicating their understanding and voluntary agreement to participate in the study.

Following the distribution phase, a structured survey questionnaire addressing participants' demographic profiles and perceptions of the program's effectiveness in implementation, relapse prevention, and social reintegration will be distributed on-site, and

the collected responses will be checked for completeness before encoding. Each item will be assigned numerical values on a four-point Likert scale. The data will be encoded and analyzed in Microsoft Excel or SPSS to ensure the accuracy of the data collected.

The following statistical tools will be used to analyze and interpret the data.

- **Frequency and Percentage**. It will address problem one by determining the demographic profile of the respondents regarding length of service and training background.
- **Weighted Mean**. It will address problem two by determining the perception of effectiveness of program implementation, relapse prevention, and social reintegration.
- **Spearman's Rho**. It will address problem three in determining the significant relationship between the respondent's profile and their perceived effectiveness on drug rehabilitation and aftercare programs.

2.5 Research Ethical Standards

In this research, entitled "The Perceived Effectiveness of Drug Rehabilitation and Aftercare Programs," several ethical considerations will be addressed to ensure that the study meets the required standards for research integrity and the protection of respondents.

A. Protection of Intellectual Property Rights (IPR)

This study upholds the ethical standards of intellectual property rights by ensuring that all research materials are correctly attributed and protected under Philippine copyright law. All outputs of the study are developed and used with full respect for original authorship and are properly cited.

B. Informed Consent

The researchers will provide a comprehensive explanation of their objectives, methodologies, potential risks, and benefits to all participating facilitators. The researchers will ensure that participation is voluntary and that all participants fully understand the information presented to them. Any concerns or questions raised by participants will be addressed promptly and transparently.

C. Data Privacy and Confidentiality

The researchers prioritize data privacy and confidentiality in this study. Participants will be informed of the research's purpose, procedures, and potential risks before providing written consent. Survey questionnaires are administered only after participants are fully informed about the study's purpose and voluntarily agree to participate. All collected data is anonymized to protect personal information, and only the research team has access to the securely stored data. All responses are kept strictly confidential and are used only for academic and research purposes. The researchers follow data privacy laws and respect intellectual property rights, which ensures participants' dignity and rights throughout the study.

D. Voluntary Participation and Freedom to Withdraw

Participation in this study was entirely voluntary for all participants. The researchers will inform the respondents of their right to withdraw at any time without negative repercussions, as stated in the informed consent forms. The researchers adhered to established research ethics guidelines, ensuring that participants

provided informed consent and understood their right to withdraw at any time, thereby highlighting the study's integrity and respect for participants' rights.

E. Minimization of Harm and Risk Management

The researchers implemented strong measures to protect human participants throughout the study. Researchers took several key steps to minimize harm to human participants, including obtaining informed consent, maintaining confidentiality, minimizing potential risks, and providing access to support resources in case of distress.

The researchers ensured that the participants in this study did not experience physical, emotional, or psychological harm by taking specific key steps.

F. Beneficence and Contribution to Knowledge

This research can contribute to both the rehabilitation center and the community by providing evidence-based insight into the effectiveness of drug rehabilitation and aftercare programs from the perspective of the facilitator. The findings can help improve program design and policy implementation. Upon request, the researcher will share the summary of the findings.

G. Justice and Fair Participant Selection

This study employed purposive sampling in selecting participants. The participants are chosen intentionally based on specific traits, characteristics, or experiences that best align with the research goals. Participating samples were selected based on clearly identifiable and fair criteria for inclusion and exclusion. The selection of samples was made in a way that ensured diversity and representation of

the facilitator population, so that no single group would be too burdened or left out of the study.

H. Data Integrity and Accuracy

The researchers present data collection and analysis using standardized methods to ensure transparency and consistency. Researchers will clearly outline the methodology, data, and results, ensuring that interpretations are based on the most accurate and reliable data available and that the data gathered are thoroughly and carefully analyzed and reviewed. The commitment to transparency not only strengthens the validity of the findings but also fosters trust and confidence among the academic community and stakeholders, ensuring the integrity of the research.

I. Transparency and Honesty in Reporting

All results, including any research limitations and findings, will be disclosed honestly and clearly in the study's report without manipulating the data. Furthermore, in accordance with ethical research practices, any copyrighted or previously published materials used will be appropriately cited and acknowledged, ensuring that the original authors and sources receive full credit. This commitment to transparency and academic honesty reinforces the integrity of the research and upholds the highest standards of scholarly conduct.

J. Use of Pretended or Copyrighted Materials

The study utilizes copyrighted or patented materials. The researchers will ask permission from the authors or rights holders before using such materials, ensuring that the data used in the study are appropriately cited. All materials will be used in

full compliance with legal and ethical standards. The study will be appropriately licensed and credited.

K. **Ethical Considerations for Animal and Human Trials**

This study prioritizes ethical considerations for human participants and does not involve animal trials. Researchers are committed to treating all participants with respect and protecting their rights and dignity throughout the research process. Adhering to strict ethical guidelines, researchers will ensure participants are fully informed about the study's objectives, risks, and benefits. This approach will provide valuable insights while maintaining the highest ethical standards.

L. Responsible Use of AI and Other Related Technologies

The study uses AI tools and digital technologies to support various stages of the research process. In this study, Grammarly and other AI-powered technologies will be used to ensure the research report adheres to proper grammar and clarity standards and to assist in structuring and summarizing preliminary findings. Ensuring that outputs are accurate and unbiased, and will be reviewed carefully and validated to ensure alignment and integrity.

M. Ethical Clearance and Institutional Approval

This study received clearance from the Ethics Review Committee of Saint Michael College of Caraga (SMCC), ensuring full compliance with the ethical standards for research. Before data collection, formal approval is also obtained from the DOH-Treatment and Rehabilitation Center in Caraga. This research adhered to key ethical principles, including respect for participant privacy, informed consent, and data confidentiality.

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